

Interview briefing

Sr. Ecommerce Manager · FSA Store / HSA Store · Christy Wood · September 14, 2026 — FSA Day

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90-second pitch

I'm Alexa Fisher, a senior e-commerce and digital operations leader at Colgate-Palmolive, based in Dallas. I turn consumer and performance insight into catalog, content, and site decisions that move conversion and mix across high-SKU environments.

Health-E built the category: the first, still-leading stores where every product is already FSA or HSA eligible, distributed through 250+ TPAs, sitting on a \$170M-scale catalog with a live personalization mandate. That is my sport — two banners, real revenue, merchandising as a P&L, not a mood board.

I'm not here to admire the mission. I'm here to own CVR, AOV, and LTV for FSA Store and HSA Store, coach a small specialist team, and make 'guaranteed eligible' also feel personally inevitable. Dallas-based, EST-available, player-coach, scoreboard-driven.

Health-E is hiring a player-coach to own two high-SKU storefronts and make personalization pay. Alexa already lives in high-SKU CPG commerce, Dallas, and insight-to-action operating rhythm. The interview is won by translating Colgate digital-shelf craft into FSA/HSA site economics — CVR, AOV, LTV — and by showing she can stand up a personalization roadmap in 90 days, not by pretending she already ran Segment.

The company in one paragraph

Tens of millions of people get a special prepaid health wallet from work. They are scared of buying the wrong thing, and FSA money evaporates every year. Health-E built the only stores where everything on the shelf is already approved — and they sit inside the benefits websites people already log into. This job is to make those stores convert like a modern retailer, not a benefits pamphlet.

H.I.G. did not buy a brochure. They bought a category captain with distribution into benefits platforms and asked it to behave like a modern retailer: personalization, private label mix, telehealth attach, conversion science. The Sr. Ecommerce Manager seat is how that mandate hits the homepage.

2,500+ Eligible catalog

250+ TPA partners

500+ Brand vendors

70M+ Addressable holders

Live store — say this

I mystery-shopped FSA Store on FSA Day. The hero is self-care, not tax. Caring Mill is running the bestseller rail. Telehealth is already a module. What I did not see is a different homepage for a deadline FSA holder versus an HSA planner — that is the 90-day job. I'd instrument holdouts in 30, segment by account type and deadline proximity in 60, and put recs on PDP and cart by 90, with incremental revenue as the only scoreboard.

One homepage for 70M wallets. Hero, seasonal rail, bestsellers, telehealth modules — same stack for a December FSA panicker and a July HSA planner. That is why this seat exists. Name it in the interview.

Caring Mill owns the bestseller rail. Private label is mix, margin, and a Children's Health Fund give-back. Your merchandising lever. Ask Susan what PL mix target is, then have an opinion.

Promo is loud this week. Sitewide 16% + FSADAY16. A conversion owner protects holdouts during a sale, or you will never know if personalization worked. Say that.

Eligibility is the brand. Every tile still has to scream 'already approved.' Never bury the guarantee for a prettier lifestyle hero.

Market

Consumer-directed healthcare is a prepaid wallet the size of a mid-size retailer, growing every year, and still shockingly bad at shopping.

The wallet. FSAs and HSAs are IRS-blessed pre-tax accounts. 2026 health FSA cap is \$3,400 (carryover up to \$680). HSAs are triple tax-advantaged and investable. Year-end 2025: 41.7 million HSAs holding ~\$174B; 35M+ FSA holders. Combined, 70M+ Americans can spend this money.

The leak. FSA is use-it-or-lose-it (rollover or Mar 15 grace — employer picks one). Billions forfeit or get blown on last-minute eligible junk. HSA holders under-spend on care and over-save. Confusion is the competitor. Education is a product.

The aisle. Eligible goods: OTC meds, pain, skin-as-treatment, sun, diagnostics, first aid, devices, menstrual, telehealth, hearing, and a long tail of 'surprising' items. Health-E's Eligibility List is a product, not a FAQ. CARES Act 2020 (OTC + menstrual without Rx) exploded the catalog.

The pipes. Third-party administrators hold the money and the login: WEX, HealthEquity, Optum, Fidelity, Lively and 250+ others. Amazon has catalog. Health-E has the benefits aisle. Jan 2026 WEX embed is the strategy in one deal.

Two banners, two jobs. FSA Store ~\$122M GMV '25, ~4.0–4.5% CVR, \$125–150 AOV — deadline heat. HSA Store ~\$48M, ~3.0–3.5% CVR, \$150–175 AOV — relationship. Combined ~\$170M. Do not average the KPIs.

The interview math. A 50 bp CVR lift on FSA Store at a ~\$9.1M month is on the order of ~\$1M extra GMV in a similar month if AOV holds — tens of millions if it annualizes. That is why this seat exists.

Where the money sits. TPA-embedded traffic is cheap acquisition. Direct/SEO/Amazon-conquest is expensive. Ask which mix they have. If TPA is fat, you are a conversion GM. If not, you are also fighting for the open tab.

Site	GMV '25	Month	Sessions	CVR	AOV
FSA Store	\$122M	\$9.1M	1.5M	4.0–4.5%	\$125–150
HSA Store	\$48M	\$4.0M	688k	3.0–3.5%	\$150–175

Competitors — why, and what to say

Amazon · High — named threat

Prime default, SKU infinity, Rufus/Alexa discovery, Subscribe & Save, improving IIAS at checkout. Lively: HSA spend on Amazon +123% YoY. Anyone who does not fear an audit starts here.

Eligibility is a filter, not a promise. No TPA embed, no Learning Center, no forfeiture-season merchandising as a brand, no Caring Mill give-back. Health-E wins on certainty and benefits distribution.

I would not out-Amazon Amazon. I would be the default eligible aisle inside the benefits login, with recs that know FSA panic from HSA planning.

CVS / Walgreens · High

Trip-based, pharmacist trust, FSA debit at POS, ExtraCare / myWalgreens, insurance adjacency, 9,000-store gravity. 'I'll grab it when I pick up a script.'

Stores optimize for scripts and trips, not planned catalog merchandising. Weak destination for a red-light mask or a \$90 device. No TPA-native storefront at this quality.

They win the urgent OTC trip. We win the planned remaining-balance shop and the TPA session.

Walmart / Target · Med-High

Omnichannel convenience, explicit FSA/HSA shops, price, pickup. Target's health aisle is culturally strong; Walmart wins rural + value.

General merch gravity. Not eligibility experts. Not TPA-native. Personalization is 'people also bought,' not 'your FSA expires in 18 days.'

Price and pickup are real. Certainty and benefits-login distribution still aren't their sport.

TPA / HSA provider shops · Med — strategic

HealthEquity, Fidelity, Lively, WEX own the login and the card. They can build a mediocre shop and still win default.

Usually poor merchandising. WEX partnership is the play: be the store inside their portal, not a rival bank. Compete on merch quality, not on who holds the HSA.

If they build a better aisle, we have a problem. Until then we are the outsourced merchandising brain.

Hims, Ro, LifeMD, LillyDirect · Med — narrative

Telehealth + GLP-1 is the growth story. LillyDirect HSA spend jumped. Patiently is Health-E's answer (GLP-1, menopause HRT, Jul 2026).

They own care. We own the eligible basket around care. Attach, don't pretend to be a clinic. Vik Panda's P&L rules matter.

Collaborate and attach on relevant PDPs. Do not homepage-hijack the store into a clinic.

Carewell & specialty medical · Med

High-consideration medical supplies, caregivers, strong conversion, content depth (incontinence, mobility).

Narrower catalog, no benefits-network distribution at Health-E's scale. Partner or compete by category — don't ignore them on PDP quality.

Steal their content depth on devices; keep our TPA pipes.

Brand.com DTC · Low-Med

Device and dermatology brands (and Amazon storefronts) sell the same SKUs at full price, with better brand sites.

They lack guaranteed eligibility + auto-substantiation + TPA traffic. Health-E is a channel they should pay to be in — merch leverage.

National brands are vendors. Caring Mill is the mix lever. Don't let a brand.com steal the remaining-balance shopper with a 20% off email.

Category-defining specialist in a market the giants treat as a payment method. Moat = guaranteed eligibility + TPA pipes + education + private label + owned telehealth. Weakness = brand awareness vs Amazon, two-site complexity, and personalization still being built — which is exactly why this seat is open.

Expert overnight

Health FSA. Employer-sponsored, pre-tax, use-it-or-lose-it. 2026 salary-reduction cap \$3,400. Optional \$680 carryover or a grace period through March 15 — employer chooses one, not both. Money is 'use or burn.' That is why FSA Store converts hotter and why Dec 31 / Mar 15 are merchandising Super Bowls.

HSA. Paired with a high-deductible health plan. Triple tax-advantaged: in, growth, out (for qualified spend). Rolls forever, investable. 2026 HSA limits sit on a different IRS table than FSA. Holders under-spend on care and over-save. HSA Store is a relationship and AOV business, not a panic business.

Dependent care FSA. Different animal. 2026 DCFSA cap \$7,500. Daycare, not bandages. Do not mix it into FSA Store merchandising. If it comes up, show you know the split.

IIAS + substantiation. Inventory Information Approval System lets an FSA/HSA debit card auto-approve eligible SKUs at qualified merchants. Health-E's guarantee is the consumer version of that: nothing on the shelf should fail substantiation. That is a legal product (Rachel Rouleau) and a merch constraint.

LMN. Letter of Medical Necessity. A clinician's note that turns a 'maybe' (massage device, some fitness, some supplements) into eligible spend. Learning Center content + PDP education is how you convert LMN shoppers without a call center.

CARES Act (2020). OTC medicines and menstrual products became eligible without a prescription. Category explosion Health-E advocated for. If they ask 'what changed the catalog,' this is the answer.

Usually eligible

OTC pain, cold, allergy, digestive; Acne treatment (not cosmetic serums); Sunscreen / mineral SPF; Menstrual products; First aid, bandages, antiseptics; Thermometers, BP monitors, pulse ox, tests; Eyeglasses, contacts, reading glasses; CPAP and many sleep-therapy devices; Hearing aids (OTC era is live); Eligible telehealth (Patiently: GLP-1, menopause HRT); Many heat/cold therapy and TENS devices

Usually not

Cosmetic skincare and anti-aging as beauty; Toothpaste, mouthwash (cosmetic oral care); Most vitamins/supplements without LMN; Gym memberships without LMN; Insect repellent (classic 'that's covered!?' miss); Diapers (usually no — exception paths exist, don't guess); General wellness gadgets with no medical claim

Guaranteed eligible is the brand. I never merch a hero that would fail substantiation. Gray-area SKUs go through compliance, or they get an LMN module, not a boost.

50 bp on FSA Store. If a recent month is ~\$9.1M at 4.25% CVR, a +0.50pt CVR move (to 4.75%) is ~+12% orders, holding AOV. That is on the order of ~\$1M extra GMV in a similar month — tens of millions annualized if it holds. This is how you talk about the job: not 'make the site nicer.'

\$10 AOV on HSA Store. HSA AOV ~\$160. +\$10 at 3.25% CVR on ~688k sessions is roughly +\$22k in a month from AOV alone. Attach, bundles, and device mix are the levers — not homepage poetry.

Forfeiture is demand. Unused FSA dollars are not a CSR problem. They are a merchandising calendar. Education in October, devices in November, kits in December, grace-period remaining-balance in March.

The job

Player-coach who owns FSA Store and HSA Store site strategy, merchandising, and personalization with CVR, AOV, and LTV on the scoreboard.

100% remote, United States — EST hours · \$120,000–\$150,000 · Discretionary, up to 15%

~~I'm passionate about wellness and omnichannel journeys.~~

I'd treat FSA Store like a deadline business and HSA Store like a relationship business, with one personalization spine and two decisioning policies.

~~I grew ecom at a Fortune 500.~~

I ran high-SKU digital execution with a conversion scorecard. Here's a test, a merch move, and the dollar effect.

~~I'd love to learn healthcare.~~

Guaranteed eligibility is the brand. Forfeiture is the demand spike. TPA embed is the moat. Personalization is the 2026 gap.

Christy Wood

Christy is not the ecommerce GM. She is the process owner: screener, scheduler, culture filter, and the person who has to sell you to Susan Elliott-Bocassi's world and whoever sits on the ecom pod. Treat her as a partner, not a gate to rush.

Warm, precise, un-flustered. She has run this process since 2022 — she can smell rehearsal and she can smell chaos. Be the candidate who makes the rest of the loop easy.

- Why this company, in two facts: guaranteed eligibility + TPA distribution, not 'I like health.'
- You can work EST, fully remote, from DFW, with overlap for NY/Dallas leadership.
- You meet the must-haves: years, SKU scale, a revenue story, some personalization, some people leadership.
- Comp in band, or a clean, adult conversation if you don't.
- You are a player-coach, not a strategy tourist.
- You did the homework — FSA Day this week, Patiently, H.I.G., two-store math.

Highest-frequency questions

Almost every screen

Tell me about yourself.

Coverage + whether you can land the plane on this job.

Use the 90-second pitch. Last sentence is the seat: own CVR, AOV, and LTV on FSA Store and HSA Store, player-coach, Dallas, EST. Do not start at college. Do not end at Colgate.

Almost every screen

Why Health-E? Why this role?

Filters tourists ('I like wellness') from operators.

Health-E invented the guaranteed-eligible aisle and sits inside 250+ TPA logins. H.I.G. is professionalizing conversion. This seat is the GM for two banners with personalization still under-built. I already live in Dallas, I already run high-SKU insight-to-action work at Colgate, and I want a scoreboard I own — not a brand campaign I decorate.

Almost every screen

Why leave Colgate / why now?

Loyalty vs hunger. Never dunk on the employer.

Colgate taught me high-SKU digital execution and a conversion scorecard. I want to own the site P&L, not just a slice of a retailer.com shelf. Health-E is the category captain in a market that is still shopping badly. Timing is FSA Day / open enrollment plus a PE growth chapter. I am not running from Colgate. I am running toward a GM seat.

Almost every screen

Walk me through a win — a time you owned a number.

Proof you are not a coordinator.

One STAR. Baseline, move, window, result, what you would repeat. Prefer CVR, mix, or revenue. Name the constraint (SKU count, legal, retailer, season). If you do not have a DTC P&L, translate digital-shelf / content / ranking work into conversion language — do not inflate.

Almost every screen

Have you managed people? How would you run 2–4 specialists?

The JD requires it. Honesty + operating system beats a fake org chart.

If you have ICs, name them and a coaching outcome. If not: DST chapter presidency, SEO, agencies, interns — then the system you'd install. Weekly scoreboard: CVR, AOV, test pipeline, merch SLA. Each specialist owns a surface (home, PLP, PDP/recs, reporting). I still approve flips. Coaching happens in the tool, not in a 1:1 monologue.

Almost every screen

This role is EST and 100% remote. You're in Dallas.

Logistics fail candidates more often than merch theory.

Central is one hour behind. 8–9a ET standups are 7–8a CT. I already live in HQ's city if they want in-person. I have worked distributed. I will not be the person who misses the East Coast morning.

Almost every screen

What are your compensation expectations?

Band is posted. Adults answer cleanly.

Posted band is \$120–150k plus up to 15% discretionary bonus, remote. I am in range for the work. I care about total: bonus, wellness and internet stipends, coaching, medical. I won't open with a number unless asked; when asked I won't theater.

Almost every screen

When can you start / what's your notice?

Colgate transitions are real.

Have the number. Typical professional notice, plus knowledge transfer. Don't be vague. Don't offer to ghost a team.

Very high

What KPIs do you watch weekly?

Highest-frequency ecom manager question. Lists revenue-only = fail.

Daily: revenue, CVR, AOV, RPV, sessions, top SKUs, errors, recs attach — FSA and HSA separately. Weekly: test pipeline, merch SLA, private-label mix, category CVR, refunds, TPA vs direct traffic. Monthly: repeat/LTV, segment ROI, flip readout, Patiently attach. I report the two or three numbers I would be fired or promoted on, not a dashboard tour.

Very high

How would you improve conversion rate here?

They are hiring a conversion owner.

Audit the funnel by banner and by landing type (TPA embed vs SEO vs email). Find the fattest leak: home relevance, PLP sort, PDP proof, cart, checkout. Hypothesis → holdout → ship. On this site I'd start with deadline-aware homepage modules for FSA and a planner homepage for HSA, then recs on PDP and cart. I would not start with a button color. A 50 bp CVR lift on FSA Store at this run-rate is millions.

Very high

How do you increase AOV without wrecking conversion?

HSA is the AOV banner; FSA will try to panic-add junk.

Thresholds (free ship is already \$50+), bundles that are medically coherent, attach on PDP (device + consumable), Caring Mill good-better-best. Guardrail: AOV tests must not drop RPV. For FSA, kits for remaining-balance spend. For HSA, higher-consideration devices and telehealth attach. Never force-add on a deadline user.

Very high

Tell me about the last A/B test you ran.

They will smell 'we launched and it felt better.'

Hypothesis, primary metric, sample, duration, call criteria, result, what you did when it lost. If traffic was thin: bigger changes, sequential tests, leading indicators (ATC, checkout start). Takeaway must have changed a rule you still use.

Very high

What's the largest catalog you've merchandised, and how do you run 2,000+ SKUs?

JD minimum: 2,000+ SKU site, 5–8 years.

Colgate is multi-category, multi-channel — I already think in assortment, ranking, and content, not a 40-SKU boutique. Operating system: taxonomy, nav, search synonyms, boost/bury rules, seasonal flips, private-label mix, kill list. Data first: internal search, zero-results, category CVR, return reasons. Taste last.

Very high

How would you approach personalization here?

Named in the JD. The gap this seat is hired to close.

30 days: instrument holdouts; map segments (FSA vs HSA, new vs returning, deadline-near, condition affinity). 60: recs on PDP and cart; homepage modules by segment. 90: ML with Mike Ng, eligibility as a hard filter, lift standards, kill rules. Scoreboard is incremental revenue vs holdout, not CTR. I will not copy-paste FSA decisioning onto HSA.

Very high

How do you prioritize the roadmap?

Player-coach with a small pod cannot do everything.

Expected incremental \$ $\times$ confidence $\times$ effort, with calendar constraints (OE, FSA Day, Mar 15, Dec 31). Compliance can veto. Marketing is an input. I keep a kill list. If it doesn't beat holdout or a merch SLA, it doesn't ship.

Very high

How do you work with marketing when they want a campaign homepage?

Seiya Vogt is the promo-calendar partner. Conflict is guaranteed.

Shared primary metric — usually RPV. Time-box the campaign, hold out a slice, pre-commit to revert. Promo calendar feeds merch; it does not veto conversion. I would rather a campaign module in a reserved slot than a full homepage hijack during a test.

Very high

How do you use analytics to increase sales?

Classic Indeed/ecom screen.

I start with the leak, not the tool. GA plus a BI layer (Looker/Tableau or Sheets if that's what's live). Segment the two banners. Watch search, sort, PDP scroll, ATC, checkout. Pair with session replay on the top five drop steps. Then one test or one merch flip — not a 40-page readout.

High

Cart abandonment is high. What do you do?

Standard CRO question. Translate to this catalog.

Split FSA panic carts from HSA research carts. Check shipping threshold messaging (\$50+ is already a lever), eligibility reassurance in cart, out-of-stocks, recs that add junk. Fix trust and completeness before discounts. A deadline FSA user needs 'this is already approved' more than 10% off.

High

If you joined tomorrow, what would you audit first?

90-day plan, compressed.

Both banners as a first-time user. Baseline CVR/AOV/RPV/recs attach/PL mix, FSA vs HSA. Stack: what's live vs slideware. Promo calendar through Mar 15. Mystery shop TPA-embedded entry vs direct. Agree the two numbers I am on the hook for. Then one flip I own.

Functional loop

How do you decide sort, boost, and bury?

This is the daily job.

Rules, not vibes: seasonality, margin, availability, eligibility, conversion, private-label mix, insight (search + returns). FSA Day and Dec 31 get deadline kits above the fold. Bury anything that fails substantiation or tanks refund rate. Review flips on a cadence Susan's world will recognize.

High

Is it more important to retain or acquire?

Classic. Wrong answer is 'both' with no math.

On FSA Store, TPA pipes already acquire. This job is conversion and remaining-balance capture — retain the relationship through the year so December isn't a fire drill. On HSA Store, LTV is the strategy: education, devices, telehealth attach. I would ask how much of traffic is already paid-for by the TPA embed before I spend like a DTC brand.

High

How do you run a peak — open enrollment, FSA Day, Dec 31?

Peak trading is the stress test.

Merch flipped before the spike, not during. Holdouts stay up or you learn nothing. Site performance, inventory on hero SKUs, Caring Mill depth, customer-service macros for eligibility. After-action within a week: what converted, what refunded, what we bury next year. I mystery-shopped FSA Store on FSA Day 2026 — promo was loud; personalization still looked like one homepage.

Very high

Tell me about a failure / a time the number disagreed with you.

Judgment and ego.

Operational, not therapeutic. I believed X, the holdout said Y, I reversed, I wrote a rule the team still uses. They need to know you will kill your own module.

High

What platforms have you used (SFCC, Builder, Segment, Looker...)?

JD name-drops. Honesty.

Name what you actually used. Then: I am operator-first. A bad experimentation culture is harder to fix than a new CDP. Tell me what's live and I'll be dangerous in it inside a month.

Functional loop

What's the difference between FSA Store and HSA Store as businesses?

If you fumble this, you are not the GM.

FSA = deadline, forfeiture, higher CVR, seasonal heat, remaining-balance kits. HSA = rolling, investable, higher AOV, education, LTV, planner psychology. Same eligibility promise, different decisioning, different merch calendar, different recs. Do not copy-paste personalization. Combined ~\$170M; FSA is the larger engine.

Functional loop

Amazon's HSA spend is exploding. Why do we win?

Lively +123% YoY. They will ask.

Amazon wins convenience and default. We win certainty (100% eligible), TPA distribution (the store inside the benefits login), education, Caring Mill margin, forfeiture-season merchandising, and now owned telehealth. Strategy: be the default eligible aisle where the card already lives — not a louder Amazon. I would never trash them. I would out-specialize them.

Functional loop

What is FSA/HSA eligible — and what isn't?

Expert overnight. Compliance is a product.

Yes: OTC meds, menstrual, sunscreen, diagnostics, many devices, eligible telehealth, hearing, first aid. No, usually: cosmetic skincare, toothpaste, most vitamins, gym without LMN, insect repellent. Gray: LMN. I merch inside Rachel's fence. The Eligibility List is a product, not a FAQ.

Functional loop

What is the TPA moat?

Distribution is why this company exists.

250+ administrators hold the login and the debit card. WEX embed in Jan 2026 is the playbook: don't fight the bank, be the store inside it. Amazon has SKU infinity. We have the benefits aisle. If TPA traffic is fat, this job is conversion. If not, it is also acquisition-adjacent — I will ask which.

High

How does Patiently change merchandising?

Jul 2026 launch. Serious candidates have an opinion.

Owned telehealth — GLP-1 and menopause HRT, FSA/HSA eligible. I treat it as attach on relevant PDPs and a journey, not a separate brochure. Carewell and Hims own clinics; we own the eligible basket around care. I'd want Vik Panda's P&L rules before I flood the homepage.

High

How do you think about Caring Mill vs national brands?

Susan's world. Mix is a merch lever.

Private label is margin, control of the shelf, and a Children's Health Fund give-back. Bestsellers already skew Caring Mill devices. I'd set a mix target, not a vibe, and never let PL bury a converting national SKU just to hit a percentage. Amazon cannot copy the give-back + eligibility bundle overnight.

High

What would you merchandize for open enrollment vs December 31?

Calendar fluency.

OE (Sep–Nov): education, 'what can I buy,' FSA Day, new-money storytelling. Dec 31: remaining balance, kits, devices, gift-of-health, sort for urgency. Mar 15 grace: last-chance for plans that allow it. Q2–Q3: evergreen AOV, PL mix, personalization, telehealth attach. I would never run one homepage through all four.

Ask them

What does a great first year look like on CVR, AOV, and personalization revenue — and what's in the way?

Scoreboard, not title.

Listen for whether they have a number. If they don't, you will be writing it.

Ask them

How do FSA Store and HSA Store share a stack versus diverge on decisioning today?

Two-banner problem.

If they say 'they're the same site with a tab,' that is your 90-day mandate.

Ask them

Which recs / CDP / experimentation tools are actually live, and who owns the model versus the experience?

Mike Ng vs this seat.

Clarify before you accept a mess.

Ask them

How is success split between TPA-driven traffic and onsite conversion?

Tells you if this is a conversion job or also acquisition.

If TPA traffic is fat, protect CVR. If not, you are also fighting Amazon for the open tab.

Ask them

What should this pod stop doing in the first 90 days?

Bias for Action.

You want them to name the graveyard.

Ask them

How does Patiently change merchandising — attach, journey, or a separate P&L?

Jul 2026. Have an opinion, then update it with theirs.

Serious candidates ask this.

Ask them

Who is the functional hiring manager after Christy, and what does a great debrief from her look like?

Help Christy sell you. Likely Susan Elliott-Bocassi.

Shows you understand process vs function.

Ask them

What is the Caring Mill mix target, and where is eligibility currently blocking tests?

Merch + compliance as a system.

You are already thinking like CMOO + CCO.

STAR scaffolds

I owned a number

ROI ownership / CVR / AOV

Pick one Colgate digital-shelf or site initiative where you moved conversion, mix, or revenue. Need: baseline, the move, the window, the result, what you would do again.

I flipped a shelf on a calendar

Seasonal + evergreen merch, sort, boost/bury, monthly flips

FSA Store is a calendar business (open enrollment, Mar 15, Dec 31). Tell a story about a timed merchandising or content change — back-to-school, oral-care season, retailer event — and how you prioritized SKUs.

I made it feel relevant

Recs, segments, personalization lift

If you have recs/CDP, use it. If not: digital-shelf relevance, audience content, retailer algorithms, CRM. Then: how you'd stand up HEC's recs roadmap.

I ran a real experiment

A/B, MVT, statistical rigor

Hypothesis, primary metric, sample, what you did when it lost. They will smell 'we launched and it felt better.'

I coached a small team

Manage 2–4 specialists

Direct reports if you have them. Else: DST chapter, SEO, agencies, interns, matrix reports. How you set a standard and still did the work.

I moved creative / UX / data without a title

Partner creative, UX, data, product

A time legal, brand, or a retailer said no. How you got to a live page anyway.

I shipped at PE speed

Bias for Action, remote, self-directed

A change that happened in days, not a planning cycle. H.I.G. is listening for this even if Christy doesn't say PE.

I was wrong

Judgment, ego, measurement

A merchandising or content call that lost. What the data said, how you reversed, what rule you now carry.

90-day plan

Days 1–30 — See the machine

1. Sit with specialists: who owns home, PLP, PDP, recs, reporting. Skills matrix, not vibes.
2. Baseline both sites: CVR, AOV, LTV, sessions, RPV, recs attach, PL mix, test velocity. FSA vs HSA separately.
3. Walk the eligibility product with compliance (Rachel's world). Know what you cannot merch.

4. Map the promo calendar through Mar 15 and next open enrollment.
5. Inventory the stack: platform, CDP, recs, A/B. What is live vs slideware.
6. Mystery shop both sites as a first-time FSA user in December and an HSA parent in July.
7. Agree the 2–3 numbers you will be fired or promoted on.

Days 31–60 — Move the needle

1. Ship the first merchandising flip you personally own. Deadline-aware if in-season.
2. Stand up a holdout for recs or homepage modules. No lift claim without it.
3. Segment v1: account type, recency, deadline proximity, category affinity.
4. Kill one underperforming module. Bias for Action is a value — demonstrate it.
5. Weekly scoreboard the team actually uses. Cancel a meeting to pay for it.
6. Partner Seiya's calendar: one campaign that does not wreck the test roadmap.

Days 61–90 — Install the operating system

1. Personalization roadmap through next two seasons, ranked by expected incremental \$.
2. ML recs working session with Mike Ng: features, cold start, eligibility constraints.
3. Caring Mill mix target — private label as a merch lever, not a side SKU.
4. Patiently attach hypothesis on relevant PDPs (GLP-1, menopause, hearing).
5. Team rituals locked: flip reviews, test readout, coaching in the tool.
6. Write the year-1 site strategy for both banners in 6 pages, not 60.

Drill answers

1. **FSA vs HSA in one breath.**
Deadline vs forever. CVR vs AOV. Panic vs plan.
2. **Name three HEC brands besides the two stores.**
Caring Mill, WellDeserved, Patiently.
3. **Who bought the company, when, and why it matters to this job?**
H.I.G., Jun 2024, growth + professionalize conversion.
4. **Who is CEO and what was he before?**
Zack Peckham, CFO, internal since 2017.
5. **Approximate FSA Store monthly CVR and AOV.**
 $4.0\text{--}4.5\% \cdot \$125\text{--}150$.
6. **Why is Amazon the named threat?**
HSA spend +123% YoY (Lively). Convenience default.
7. **What is the TPA moat?**
250+ admins. WEX embed. Demand inside benefits login.
8. **What happened Jul 2026?**
Patiently telehealth — GLP-1 + menopause HRT.

9. What is FSA Day?

Sep 14–16 2026, 16th birthday, open enrollment heat, FSADAY16.

10. Who is Christy Wood?

TA partner since 2022, SC. Process owner, not ecom GM.

11. Name the CMOO and why she matters.

Susan Elliott-Bocassi — merch/ops gravity.

12. 2026 health FSA cap and carryover.

\$3,400 · \$680 carryover (or Mar 15 grace, not both).

13. Name three things that are usually NOT FSA eligible.

Cosmetic skincare, toothpaste, most vitamins, gym without LMN, insect repellent.

14. What is a holdout and why do you keep it up during FSA Day?

Control group. Without it you cannot claim personalization lift.

15. What is IIAS?

Inventory Information Approval System — auto-substantiation at the SKU.

16. 50 bp CVR on FSA Store — why say it?

Tens of millions annualized if it holds. That's the job, not prettier tiles.

17. Who owns compliance / eligibility as a product?

Rachel Rouleau, Chief Compliance Officer.

18. Your 90-second last line.

Own CVR, AOV, LTV. Player-coach. Dallas, EST.